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Western Medical Oppression of the Asian, Female, and Impaired Bodies

Twelve million adults are misdiagnosed every year in the United States (Szabo). Women, racial, and ethnic minorities are 20-30% more likely to be misdiagnosed compared to white men (Szabo). As a result, 795,000 patients are permanently disabled or die every year due to misdiagnosis (Szabo). I will refer to “disabled people” as having impairments, following the definitions of impairments and disability by Rosemarie Garland-Thomson in “Misfits: A Feminist Materialist Disability Concept.” Garland-Thomson defines impairments as “bodily states or conditions taken to be impaired” and a disability as “the social process of disablement that gives meaning and consequences to those impairments in the world” (Garland-Thomson). The distinction between impairments and disabilities is distinctly articulated in Vivian Chong’s graphic memoir *Dancing After TEN*. Vivian develops a disability; however, it is society—not her blindness—that disables Vivian based on her race, gender, and physical impairment. Chong illustrates the intersectionality of race, gender, and disability—specifically Asian women with impairments—through the abusive medical treatment she receives.

Medicine and medical research have historically preyed on non-white women with impairments. This exploitation is driven by racial and gendered biases that lead to harmful medical practices. For several years in the late 1840s, James Marion Sims, a white man bestowed with the title “the father of modern gynecology,” performed dozens of excruciating experimental surgeries on forcibly enslaved Black women without their consent (Kuta). Three of these women, Anarcha, Lucy, and Betsey, suffered from vaginal fistulas, which prevented them from being unable to perform as much physical labor as was demanded of other forcibly enslaved Black people (Kuta). Although their impairments are not equivalent to impairments modern society labels “disabilities,” these women—based on Garland-Thomson’s definition of disabilities—were disabled by their white enslavers, who used their race, gender, and impairments as a means to incite medical abuse. Anarcha, Lucy, Betsey, and the other forcibly enslaved Black women who were experimented on were not abused because they were Black, or because they were women, or because they had impairments. They experienced medical abuse because they were all three. It is indefensible to justify the singular societal label of race, gender, or physicality as “necessary” in the 1840s to “approve” the experimentation of these women. Two other well-known instances of erasures of the contributions of Black people (forcibly, unknowingly, and without compensation through coercion or intentional omission) to the development of medical research include the Tuskegee syphilis study and Henrietta Lacks’ cells (“Racism in Medicine’s”). The reality is, only if these people were white, male, and able-bodied would they never have been considered for exploitation.

In haunting a parallel to the experiences of the Anarcha, Lucy, and Betsey, nearly two hundred years later, medical neglect and abuse are experienced by Vivian during her hospital stay. During the first of many hospital scenes, three doctors examine Vivian’s eyes before

surgery. Despite her crying and expressing verbal discomfort with the examination, none of the doctors ask her how she feels or for permission to proceed with the examination (Chong, 3). When Vivian is bedridden at the hospital with TEN (toxic epidermal necrolysis), she experiences various forms of neglect and abuse by multiple medical care providers. One walks past a sobbing Vivian claiming, “I do not understand your English” (Chong, 20). Yet, in moments of distress, when have tears not been a universal language? The provider’s actions reveal how Vivian’s race and impaired state are exploited to assert power over her, further subjugating her in a vulnerable moment. By not providing or becoming a source of knowledge, mobility, or independence, she is culpable in creating Vivian’s disabled state, oppressing Vivian to benefit herself. Later, a nurse yells, “Look at what you’ve done! Not only have you set off the machine, you wet your bed with all the ice cubes!” (Chong, 39). Another nurse condescends to her, saying, “Count sheep. You know how to count, right? Don’t make a fuss, now!” (Chong, 44). In both scenarios, the nurse infantilizes Vivian; their comments illustrate how people with impairments are not seen as full participants in society. Their dismissal of Vivian’s discomfort suggests that her suffering is a nuisance not legitimate enough to warrant empathy or adequate care. In all of these instances, medical care providers abuse their power over Vivian, mistreating a patient entirely dependent on their care. They reflect broader social attitudes that reinforce Vivian as an object, not a person.

In what is arguably the “worst” offense in Vivian’s objectification, a male nurse takes Vivian outside to show off his car, suggests there will be future similar interactions with each other, and, while showering her, inappropriately touches her breast (Chong, 53). Unlike the overwhelming number of graphic novels and comic books that oversexualize and objectify women, *Dancing After TEN* never promotes Vivian as a sexual figure. Its focus is on Vivian “the person,” not Vivian “the woman,” with an impairment. Perhaps that is why her sexual assault scene is so unsettling. Although her initial interactions with the nurse—going inside his car and his comments—seem slightly strange, his actions do not seem to warrant immediate action against him. It is not until he touches Vivian and she internally yells “NO” does the realization hit the reader that this nurse has abused his position as both medical care provider and able-bodied person to sexually assault a patient physically restrained by her body. But it is these unsettling experiences that are normalized shadows in the lives of billions of girls and women. For those who do not identify as girls or women or female-presenting, the nature of Vivian’s assault can be difficult to understand. Vivian is sexually assaulted. As defined by RAINN—Rape, Abuse & Incest National Network—sexual assault is “sexual contact or behavior that occurs without explicit consent,” including “[f]ondling or unwanted sexual touching” (“Sexual Assault”). Because of the power imbalance and nature of the assault, Vivian was not in a safe position to remove herself from the situation. She attempted to move to safety by claiming she was cold, so he would take her back to the hospital (Chong, 53). Vivian has the added disability of being physically restrained by her body. Because the emotional impact of assault is often dismissed or minimized and the legal system requires more than discomfort to pursue legal action, even if Vivian were to take legal action against the nurse, he would likely be found innocent; furthermore, he can argue the nature of his job requires him to have physical contact with patients like Vivian, thus it is unlikely he would receive even a reprimand from the

hospital. In medical settings, complaints from women are often ignored, deemed “emotional,” “attention-seeking,” and “overreacting.” Women are the predominant sufferers of chronic diseases that start with pain because medical care providers fail to seriously consider women’s initial complaints of pain (Cleghorn). “Women are more likely to be offered minor tranquilizers and antidepressants than analgesic pain medication. Women are less likely to be referred for further diagnostic investigations than men are. And women’s pain is much more likely to be seen as having an emotional or a psychological cause, rather than a bodily or biological one” (Cleghorn). Moreover, Vivian, like so many other sexual assault victims, likely does not have the resources or support to take legal action against her attacker. This thought process is what girls and women must constantly think about and contend with every day. In cruel irony, the scene of Vivian’s assault is what makes Vivian the character more akin to other female characters in comics. Vivian is forcibly sexualized by an outside force posing as a means of her empowerment. Comic artists created strong, powerful heroines under the guise of feminism, but they depicted these women as sexual objects. Thus, empowerment is an illusion masking objectification. As comics depict female strength through a sexualized lens, medical systems have historically masked abuse as part of treatment to empower. In both cases, true intent is reinforcing power structures that disempower women.

Historically, the medical system has systematically excluded women from clinical research, resulting in harmful health outcomes and death. In 1977, the FDA (Food and Drug Administration) created a guideline to exclude women with “reproductive potential” from participation in early clinical research—this initiated a trend of excluding most, if not all, women from medical research (Alvidrez). It was not until 1993 that the United States Congress passed a law requiring women in clinical trials sponsored by the NIH (National Institutes of Health) (Alvidrez). However, the repercussions of the FDA’s 1977 guidelines endure; a 2020 study from UC Berkeley and the University of Chicago found that women experience adverse effects of drug medications at twice the rate of men (Zucker). These emissions in research have compounded disparities in care for women, particularly those who also face racial and physical impairments.

People of Asian descent living in the United States have a history of being denied adequate medical care. Since the mid-1800s, people of Asian descent living in the United States have been used as scapegoats for public health crises—a trend that continues today, where Asian Hate has been revamped following the COVID-19 pandemic (Lee). In the late 1800s in San Francisco during the smallpox epidemic, health officials ordered “the fumigation of every house in Chinatown” (Lee). In 1899, when a Chinese man was diagnosed with the bubonic plague, Chinatown was immediately placed under military quarantine and residents were subjected to “dehumanizing treatment at disinfection stations, including being stripped naked, fumigated and physically inspected in public;” their homes sprayed with chemicals and then burned in “controlled” fires that ultimately grew out of control and decimated Chinatown; and finally being marched to detention camps under guard for weeks (Lee). The limited “medical care” they received was intentionally dehumanizing and humiliating. In San Francisco, the plague was traced to an Australian man who arrived on a ship, but because the first person to contract it was

a Chinese immigrant, blame was immediately placed on the Chinese community (Hunter). The government shut down Chinatown and prevented Chinese residents and food from leaving—white residents were free to come and go (Hunter). The government denied the outbreak of the plague, leaving “at least 280 infected and at least 172 people dead” (Lee). Chinese residents faced discrimination in hospitals: a refusal of admittance, longer wait times, their concerns were less likely to be taken seriously, and higher taxes and fees—many of which still occur today (Lee). A national survey on the healthcare of Americans of Asian descent found Americans of Asian descent were more likely to report their doctors “did not listen, spend as much time, or involve them in care decisions as much as they wanted” and less likely to report their doctors “ever talked to them about lifestyle or mental health issues” (Ngo-Metzger Q). Additionally, in light of the COVID-19 pandemic, multiple medical healthcare providers of Asian descent have spoken about their experiences of verbal and physical harassment by the people whose lives they save (Jan). As both patients and medical healthcare providers, the livelihoods and lives of Asian people continue to be devalued and discarded.

Vivian's abuse can not be attributed to any one factor—her gender, race, or physical impairment—but rather to the intersection of these identities. As a woman with Asian features and a physical impairment that confines her to a hospital bed, Vivian becomes vulnerable to the exploitation of those who are tasked with her care. Throughout her interactions in the hospital, Vivian's impairment, including confusion, diminished vision, and bodily restriction, limits her access to the resources that could offer clarity or mobility. Medical care providers are expected to provide this, yet Vivian's hospital experience is marred by systemic indifference, racial discrimination, belittlement, and sexual assault. This failure to uphold basic medical ethics reflects the profound consequences of institutional neglect.

News of medical malpractice, patient abuse, and cover-ups at the “highest” levels of the healthcare system makes for shocking headlines. While it is concerning that this occurs at our “highest” levels, what should concern us more is the discrimination among individual medical care providers who foster this treatment. Grace, the Chinese caretaker hired by Vivian's mother, exemplifies this issue. Tasked with helping Vivian navigate life with blindness, she instead becomes a source of emotional and physical strain. Within seconds of her introduction, Grace interrupts Vivian and says her home is a mess; she constantly yells at Vivian, bluntly orders Vivian not to throw up when she says she feels sick, refuses to slow her walking pace, brings Vivian along to eat with her friends then tells her not to make a mess while eating because it makes her friends uncomfortable, and complains about having to wash Vivian's blood off the carpet when her period finally starts (Chong, 63-66). As Vivian's caretaker, Grace is responsible for supporting Vivian while she adjusts to life with blindness. But she subjects Vivian to emotional and physical abuse in her own home—belittling her, disregarding her needs, and creating an uncomfortable environment. This reflects the reality for many individuals with impairments; medical care providers entrusted with the well-being of their patients perpetuate harm rather than providing the life-saving support vulnerable people need.

It is also fascinating how, despite sharing both race and gender with Vivian, Grace exhibits some of the most invasive and insulting attitudes of the medical care providers who

interact with Vivian. This underscores the painful truth that even when individuals share certain aspects of identity, they can still be marginalized or mistreated due to another societal label. The idea that race and gender might act as shields falls apart when met with the reality of how intersecting identities can still result in exclusion, demonstrating how no one is immune to an oppressive system built to neglect and exploit vulnerable bodies.

Grace's treatment of Vivian also alludes to the general attitudes of Asian culture towards mental health services. In addition to the physical disablement patients experience in the healthcare system, people with impairments—particularly those who develop impairments later in life—grapple with mental health-related issues of shame, guilt, blame, and resentment. The article "Guilt, Shame and Embarrassment: Revelations of Face and Self" explains that guilt can occur "when performance fails to meet expectations, especially when personal responsibility is involved" (Ho). When Vivian's mom asks if her caretaker is helpful, Vivian never mentions Grace's mistreatment. Her mom explains that they "only have enough money to pay [Grace] for two more weeks" before asking if Vivian will be okay by herself afterward; to which Vivian responds affirmatively (Chong, 71). In Asian culture, familial obligation is a predominant and often overbearing aspect of life, as many people of Asian descent grapple with appeasing cultural norms. A study on familial assistance among youths defines familial obligation as the "psychological sense that one should help, respect, and contribute to their family" (Telzer). An article on guilt in Asian students summarizes the previously mentioned study, stating, "Asian youth spend more time assisting family than their White counterparts, which provides psychological benefits, such as a sense of purpose or stronger familial connection" (Covarrubias). In *Dancing After TEN*, the roles are reversed: Vivian becomes temporarily reliant on the care of her mother and aunt. Although it is never explicitly stated that Vivian feels like she is burdening her family, reading between the lines, or rather the panels, insinuates Vivian's guilt for unintentionally demanding more of her mother. Vivian likely feels guilty for being unable to carry out her assumed personal responsibility to her family, caring for her elders. Additionally, we glimpse how the disablement of Vivian inhibits her self-esteem after her boyfriend Michael leaves her, and she sits on the floor hugging herself, sobbing, thinking, "I'm sorry...I'm sorry I didn't get better sooner...I'm sorry all of this happened. Now I really am alone..." (Chong, 80). Despite not being at fault for her developed impairment or her recovery time, Vivian's apology implies she feels guilty for Michael's loss of romantic love for her. She blames herself for her impairment and not being able to recuperate faster. In the panels containing these thoughts, the background is light blue. Throughout the novel, Chong colors solely in either black or blue. In comics, blue conveys trust, confidence, and wisdom (McLachlan). Superman and Captain America are iconic superheroes in blue, most coveted for their reliability, loyalty, and unwavering morality. While black is a base color for the majority of the drawings, blue is used as an accent color to not only draw the reader's eye to particular objects and people but also to emphasize emotional moments and Vivian's inner thoughts. Although many English speakers associate blue with sadness, most people think of blue as "a very positive color associated with calmness, contentment" ("Red with anger"). Chong's consistent use of blue and the wide range of emotions and states of being associated with it demonstrate how Vivian adapts and develops

emotionally while simultaneously maintaining her identity. Vivian's thoughts are not absolutely unique to her; many people with impairments, particularly those who develop impairments later in life, have similar emotions and thoughts. When Vivian tries a blindness support group, the scene opens with an entirely blue page with black figures expressing thoughts such as "God is punishing me," "What kind of life is that?," "What did I do to deserve this?," "The world just isn't fair to good people!," and "How is that just?" (Chong, 83). The support group vocalizes guilt and resentment for having vision impairments. In blue thought bubbles, Vivian thinks, "This is so depressing! Why would it help me to hear other people be miserable? We are blind, not dead!," before deciding the group is not for her and leaving (Chong, 84-85). Although Vivian discovers her own methods of adjusting to life with an impairment, her decision to reject a mental health service is a common practice for many people of Asian descent. In Asian cultures, there is a stronger "negative association with mental health help-seeking behaviors" than in Western cultures (Maeshima). "Asian cultural values stress the importance of restraining emotions, avoiding shame, and saving face. These values may conflict with self-disclosure and emotional expression, which are encouraged in traditional forms of counseling in America. Due to stigma toward professional mental health help-seeking in Asian cultures, seeking mental health services can be a source of shame in the family system" (Maeshima). It is likely that because of her Asian heritage, Vivian is subconsciously adverse to or wary of actively seeking mental health support systems. Western healthcare systems are not educated on the stigma of mental health that surrounds Asian culture; hence, they are not equipped to attract patients of Asian descent nor appropriately care for their convoluted hesitancy in addressing mental health. A survey on the accessibility of mental health services for Asian American women with histories of depression and suicide found the "majority (82 %) of interview participants identified service level factors influencing help-seeking behaviors, including the failure of professionals to understand and acknowledge the stigma surrounding mental health concerns in the Asian community, the lack of a more holistic approach, and the limited number of dual-culture practitioners" (Augsberger). The stigma surrounding mental health in Asian communities hinders access to care for women and perpetuates cycles of silence and suffering, illustrating the need for culturally sensitive interventions. The Asian "family system" is built upon the collectivistic culture ingrained in most Asian societies. In a collectivistic culture, individuals are defined by group identity, and the "autonomous self is deemphasized" (Kawamura, 95). One's physical appearance does not represent simply themselves, but their family and Asian community; because of this, "those with body types, physical appearances, or physical disabilities that deviate too far from the norm may experience not only their own disappointment but also the disapproval of their family or community" (Kawamura, 95). This intensifies the emotional struggles resulting from the cultural pressures placed upon disabled Asian women, making them far less likely than their male able-bodied counterparts, male impaired counterparts, and female able-bodied counterparts to seek help.

Western institutional medical systems were created to serve and empower white people, men, the able-bodied, and the heteronormative; all others are vulnerable to medical malpractice because medical institutions continue to neglect other social groups. A system designed to

provide care for all neglects and mistreats those who are at the most risk—the non-white, women, the impaired, and the queer. To address systemic injustices within healthcare, we must challenge institutionalized discrimination and individual bias, demanding more inclusive, empathetic, and equitable treatment for people marginalized by social constructs to create a medical system that recognizes the dignity of every patient.

Works Cited

- Alvidrez, Jennifer et al. "The National Institute on Minority Health and Health Disparities Research Framework." *American journal of public health* (1971) 109.S1 (2019): S16–S20. Web.
- Augsberger, Astraea et al. "Factors Influencing the Underutilization of Mental Health Services among Asian American Women with a History of Depression and Suicide." *BMC health services research* 15.1 (2015): 542–542. Web.
- Chong, Vivian, and Georgia Webber. *Dancing after Ten*. Fantagraphics Books, Inc, 2020.
- Cleghorn, Elinor. "The Long History of Gender Bias in Medicine." *Time*, Time, 17 June 2021, time.com/6074224/gender-medicine-history/. Accessed 5 Nov. 2024.
- Covarrubias, Rebecca et al. "Facets of Family Achievement Guilt for Low-Income, Latinx and Asian First-Generation Students." *Cultural diversity & ethnic minority psychology* 27.4 (2021): 696–704. Web.
- Garland-Thomson, Rosemarie. "Misfits: A Feminist Materialist Disability Concept." *Hypatia*, vol. 26, no. 3, 2011, pp. 591–609., doi:10.1111/j.1527-2001.2011.01206.x.
- Ho, David Yau-Fai, Wai Fu, and S. M Ng. "Guilt, Shame and Embarrassment: Revelations of Face and Self." *Culture & psychology* 10.1 (2004): 64–84. Web.
- Hunter, Brittany. "America's Tragic History of Discrimination against Asian-Americans." Pacific Legal Foundation, 22 Apr. 2021, pacificlegal.org/americas-history-of-asian-discrimination/. Accessed 3 Nov. 2024.
- Jan, Tracy. "Asian American Doctors and Nurses Are Fighting Racism and the Coronavirus ." *The Washington Post*, 19 May 2020, www.washingtonpost.com/business/2020/05/19/asian-american-discrimination/. Accessed 3 Nov. 2024.
- Kawamura, K.Y. "Body Image among Asian Americans." *Encyclopedia of Body Image and Human Appearance*, edited by Thomas Cash, Academic Press, 2012, pp. 95-102. doi.org/10.1016/B978-0-12-384925-0.00039-0.
- Kuta, Sarah. "Subjected to Painful Experiments and Forgotten, Enslaved 'Mothers of Gynecology' Are Honored with New Monument.", Smithsonian Institution, 11 May 2022, www.smithsonianmag.com/smart-news/mothers-of-gynecology-monument-honors-enslaved-women-180980064/. Accessed 25 Oct. 2024.
- Lee, Michael. "When Chinese Americans Were Scapegoated for Bubonic Plague." *History, A&E Television Networks*, 11 Oct. 2022, www.history.com/news/bubonic-plague-honolulu-fire-san-francisco. Accessed 3 Nov. 2024.
- Maeshima, Lindsey S, and Mike C Parent. "Mental Health Stigma and Professional Help-Seeking Behaviors among Asian American and Asian International Students." *Journal of American College Health* 70.6 (2022): 1761–1767. Web.
- McLachlan, Brian, and Aaron Hanson. "Superhero Color Theory, Part I: The Primary Heroes." *ComicsAlliance*, 2 June 2016,

- comicsalliance.com/superhero-color-theory-primary-heroes/. Accessed 6 Nov. 2024.
- Ngo-Metzger Q, Legedza AT, Phillips RS. Asian Americans' reports of their health care experiences. Results of a national survey. *J Gen Intern Med*. 2004 Feb;19(2):111-9. doi: 10.1111/j.1525-1497.2004.30143.x. PMID: 15009790; PMCID: PMC1492145.
- “Racism in Medicine’s Inauspicious Moments – a Brief Timeline.” The University of Arizona Health Sciences, 1 Feb. 2022, healthsciences.arizona.edu/connect/racism-medicines-inauspicious-moments-brief-timeline. Accessed 3 Nov. 2024.
- “Red with anger or feeling blue? The link between color and emotion, with Domicile Jonauskaite, PhD.” *Speaking of Psychology* from American Psychological Association, Aug. 2023, www.apa.org/news/podcasts/speaking-of-psychology/color-emotion. Accessed 2 Nov. 2024.
- “Sexual Assault.” RAINN, rainn.org/articles/sexual-assault. Accessed 4 Nov. 2024.
- Szabo, Liz. “Medical Errors Kill Scores Each Year in the U.S., Especially Women and Minorities.” NBC News, NBCUniversal News Group, 15 Jan. 2024, www.nbcnews.com/health/health-news/medical-mistakes-are-likely-women-minorities-racial133726. Accessed 5 Nov. 2024.
- Telzer, E.H., Fuligni, A.J. A Longitudinal Daily Diary Study of Family Assistance and Academic Achievement Among Adolescents from Mexican, Chinese, and European Backgrounds. *J Youth Adolescence* 38, 560–571 (2009). <https://doi-org.uoregon.idm.oclc.org/10.1007/s10964-008-9391-7>
- Zucker, Irving, and Brian J. Prendergast. “Sex Differences in Pharmacokinetics Predict Adverse Drug Reactions in Women - Biology of Sex Differences.” BioMed Central, BioMed Central, 5 June 2020, bsd.biomedcentral.com/articles/10.1186/s13293-020-00308-5.

(Works Cited and Advanced Labor Works Cited have been combined into Works Cited.)